Vemurafenib alone — advanced melanoma

The treatment this guide covers This is vemurafenib monotherapy. It is not the cobimetinib combination and has no cobimetinib seven-day break. The team must plan any radiotherapy because BRAF treatment can increase radiation-related reactions.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Vemurafenib alone — advanced melanoma · support plan

Active medicines: Vemurafenib

Emetogenic risk: Minimal per National Cancer Control Programme (NCCP) 00102 v6; do not replace with the combination classification

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- This is a minimal-emetic-risk oral regimen in the NCCP plan. Anti-sickness medicine is chosen to match symptoms and interactions; do not add over-the-counter medicines without checking. Agree a hydration and diarrhoea plan, and report inability to drink or persistent symptoms.
- Blood count, kidney and liver tests before each cycle. NCCP specifies electrocardiogram (ECG) every four weeks for the first 12 weeks, then every 12 weeks and after dose changes. Its skin-review row states week eight then every 12 weeks; current Zelboraf product information instead requires monthly prescriber skin examinations. The team should supply a reconciled written skin-check plan using the current product information.
- Contact the oncology team promptly for fainting or palpitations, eye pain or blurred vision, a blistering or rapidly spreading rash, severe sunburn, persistent vomiting or diarrhoea, reduced urine or yellow skin or eyes. Report new or changing skin growths even when the melanoma is responding.
- Use effective contraception during treatment and for at least six months after the final dose. Vemurafenib may reduce the effectiveness of hormonal contraception, so ask for an appropriate alternative or additional method. Discuss fertility and breastfeeding before treatment; tell the team immediately if pregnancy is possible.
- This is vemurafenib monotherapy. It is not the cobimetinib combination and has no cobimetinib seven-day break. The team must plan any radiotherapy because BRAF treatment can increase radiation-related reactions.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00102 v6 · NCCP 00102 v6 supportive-care section; current product information for medicine-specific precautions

Sources checked 2026-11-06 · SACTCheck v0.84.42 · Regimen-level drafts; prescriber and oncology-pharmacy verification required